YG Critical Care LocSSIP:

Pleural Tap / Intercostal Chest Drain V1.1

Procedure date/time:	
Operator(s):	
Bedside nurse/assistant(s):	
Supervised? (Name/grade):	

Addressograph

Name

Hospital Number

Date of Birth

Completion of this form is a joint responsibility between operator and bedside nurse/assistant

BEFORE THE PROCEDURE

Indication (e.g. pneumothorax/effusion/empyema):

Appropriate consent and ID check:	☐
Drain selected and available	☐
Ultrasound and sterile cover available	☐
Review of imaging: CXR/CT/Pleural US	☐
Site of insertion identified and marked	☐
Medicines and coagulation checked	☐
Allergies checked	☐

TIME OUT

Confirmation between team members before procedure

Team members introduced and roles assigned	☐
Side confirmed with imaging and marking	
LEFT **RIGHT**	
Position optimal	☐
Adequate sedation/paralysis	☐
Adequate ventilator settings and FiO_2	☐
Any concerns?	**STOP**: GET HELP

(If bedside USS done) Effusion Depth (cm):

Other findings:

During the Procedure

Sterile gloves/gown/hat/mask worn by Operator(s)	☐
2 applications of 2% Chloraprep	☐
Sterile field maintained with large fenestrated drape	☐
Sterile sheath/gel used with ultrasound probe	☐
Local anaesthetic used (document in medication chart)	☐
STOP if unable to aspirate air or fluid at measured depth	☐
2x50ml Samples taken (see overleaf for guide)	☐

Drain size

Routine: 12fr ☐

Trauma/septae/ empyema: 18Fr ☐

Drain site & depth:

Left ☐ Right ☐

Technique used:

Volume initially drained (max 1500mls):

SIGN OUT

	N/A	Y
Tube secured with sutures/dressing	☐	☐
Sharps disposed of		☐
Handover to nursing staff inc. max. drainage 1500ml in 1st hour, then max 500ml/hr until less than 500ml/hr	☐	☐
Chest x-ray ordered & reviewed (or N/A if uncomplicated)	☐	☐
Drugs/LA documented in medication chart (including 20ml QDS flushes)		☐
Samples labelled and sent to lab (see back page for guide and record of results)		☐
Adverse events documented in notes and Datix submitted (or none)		☐
Chest Drain observations started [see overleaf for detail]	☐	☐
Kit and trolley cleaned, re-kitted and stored; low stock items re-ordered		☐

Signature of responsible operator:

Print: GMC/NMC:

Guide to samples

Send pleural samples for:

- Blood gas syringe (unless loculated, echogenic on US or sample cloudy/visible debris) for pH & glucose and haematocrit
- White universal container **plus** pair of culture bottles (8ml each) for WCC/RCC/MC&S [to Microbiology]
- 2-5ml in white universal container Protein/LDH/pH [to Biochemistry}
- 50ml for Cytology in white universal container [to Histopathology]

Also need to send serum (blood) samples to Biochem:

- Protein
- Glucose
- LDH

See guideline for samples required in: pancreatitis, oesophageal injury, chylothorax, TB, Fungi.

See separate 'Pleural Guideline' on department website for:

- Indications for pleural aspiration and proceeding to chest drain
- Guide on equipment and drain size
- Non-routine samples
- Aftercare
- Removal criteria

Guide to observations (as per ARNS Chest drain management resources)

- **Continuous direct observation** for the first 15 minutes after drain opened
- Record observations every 15 minutes for first hour, then hourly.
- On ICU observations chart, use 2 of the observation rows for chest drain observations:
  - One row—document drain is swinging/bubbling hourly
  - Next row—document amount of fluid drainage hourly
  - Do not include QDS 20ml flushes in fluid balance/drain output (complicates decision to remove).
- NB: close drain and inform medical team if **Red Flags**:
  - >1500ml fluid drained in first hour
  - >Chest pain/discomfort on drainage
  - persistent cough, worsening SOB or vagal symptoms
  - Deteriorating NEWS score or ICU observations

Test Result	Requested?	Serum result	Pleural Result
Culture	Y / N		
pH	Y / N		
Glucose	Y / N		
Protein	Y / N		
LDH	Y / N		
Cytology	Y / N		
Other:	Y / N		

Feed back on this form!
We want to know what you think
Scan the QR code to feedback

Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg